NCCP 00243 · clinician draft

Crizotinib monotherapy

Adult advanced NSCLC · ALK or ROS1 indication confirmed Adults with advanced ALK-positive or ROS1-positive NSCLC. NCCP 00243a is previously treated ALK disease; 00243b is ROS1 disease; 00243c is first-line ALK disease. Confirm the exact indication and local access; a listed indication, licence and reimbursement are different facts. This guide does not cover paediatric lymphoma or inflammatory myofibroblastic tumour.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Crizotinib daily oral treatment

Active medicines: crizotinib

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Individually selected oral antiemetic prophylaxis	Prescriber/pharmacy to select and prescribe under local oral-SACT policy	Daily during oral treatment, reviewed for ongoing need and tolerability NCCP V6 Table 5 supports daily oral 5-HT3 antagonist prophylaxis for moderate-high oral risk; crizotinib can prolong QT and slow heart rate. Resolve ECG/ electrolyte and interaction risks before selecting the agent/dose. Do not copy a parenteral Day 1–4 steroid/NK1 regimen.

Additional phase-specific support

- Crizotinib has moderate-to-high oral sickness risk. Agree daily prevention and rescue treatment with your team. The chosen medicine must account for heart-rhythm and drug-interaction risks; do not add anti-sickness medicines or a prolonged steroid course yourself.
- Avoid grapefruit/grapefruit juice and St John’s wort. Check prescription medicines, pharmacy medicines, vitamins and herbal products before taking them. Some antibiotics, antifungals, heart medicines and seizure treatments interact. Do not stop a prescribed medicine without a plan.
- Swallow 200 mg/250 mg hard capsules whole with water, with or without food; do not crush, dissolve or open them. Other formulations have different instructions. If a dose is missed, take it when remembered only if the next dose is at least 6 hours away; otherwise skip it. Never double a dose. If you vomit after a dose, do not take an extra dose; take the next dose at its usual time. Request repeat supply early and keep a current medicines list. A clinician-held dose needs clinician authorisation to restart.
- Discuss fertility preservation before treatment. Use an effective agreed contraception plan during treatment and for at least 90 days after the last dose; this applies to men and women. Oral contraceptives may be less effective. Tell the team promptly about pregnancy and avoid breastfeeding during treatment.
- Report visual changes. Take care with driving or machinery if vision, dizziness or fatigue affects safety. Avoid prolonged sun exposure and use protective clothing and sunscreen.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00243 v7 · pNCCP00243 p4; classification Tables 5/8; NCIS V8 p4 QT caution · NCCP oral SACT classification V6 and NCIS V8

Sources checked 2026-10-29 · SACTCheck v0.84.24 · Regimen-level drafts; prescriber and oncology-pharmacy verification required